

Question 1

NOT COMPLETE

Case Study Screen 1 of 6

The following 6 questions refer to this clinical scenario: NURS 1017 Unit 5 Case Study 8.

Phase 1 — Emergency Department, 2140.

The nurse reviews the ED flowsheet. **Highlight** the findings that require **immediate** follow-up.

ED Flowsheet

| Finding                 | Result                                                                                                 |
|-------------------------|--------------------------------------------------------------------------------------------------------|
| Temperature             | 35.6 °C                                                                                                |
| Heart rate              | 116/min                                                                                                |
| Blood pressure          | 96/58 mm Hg lying; 80/50 mm Hg standing, with dizziness                                                |
| Respiratory rate / SpO2 | 18/min / 97% room air                                                                                  |
| Urine output            | 100 mL of dark amber urine in the past 8 hours                                                         |
| Skin                    | Bright erythema with scaling and peeling over approximately 90% of the body, including palms and soles |
| Nails                   | Pitting and ridging of several fingernails                                                             |
| Potassium               | 3.1 mmol/L (3.5–5.0)                                                                                   |
| Magnesium               | 0.62 mmol/L (0.70–1.00)                                                                                |
| Creatinine              | 132 µmol/L (1.49 mg/dL); 84 µmol/L three months ago                                                    |
| Albumin                 | 26 g/L (2.6 g/dL) (35–50 g/L)                                                                          |
| ESR                     | 48 mm/h (0–20)                                                                                         |
| Hemoglobin              | 124 g/L (135–175)                                                                                      |
| ALT                     | 34 U/L (7–56)                                                                                          |

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Question 1

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Case Study Screen 1 of 6

The following 6 questions refer to this clinical scenario: NURS 1017 Unit 5 Case Study 8.

Phase 1 — Emergency Department, 2140.

The nurse reviews the ED flowsheet. **Highlight** the findings that require **immediate** follow-up.

ED Flowsheet

| Finding                 | Result                                                                                                 |
|-------------------------|--------------------------------------------------------------------------------------------------------|
| Temperature             | 35.6 °C                                                                                                |
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| Urine output            | 100 mL of dark amber urine in the past 8 hours                                                         |
| Skin                    | Bright erythema with scaling and peeling over approximately 90% of the body, including palms and soles |
| Nails                   | Pitting and ridging of several fingernails                                                             |
| Potassium               | 3.1 mmol/L (3.5–5.0)                                                                                   |
| Magnesium               | 0.62 mmol/L (0.70–1.00)                                                                                |
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| ESR                     | 48 mm/h (0–20)                                                                                         |
| Hemoglobin              | 124 g/L (135–175)                                                                                      |
| ALT                     | 34 U/L (7–56)                                                                                          |

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CORRECT

Score: 9 / 9 points

Rationale:

Question 2

NOT COMPLETE

Case Study Screen 2 of 6

The nurse is caring for a 54-year-old male client with plaque psoriasis progressing to generalized exfoliative dermatitis (erythroderma).

History and Physical

Nurses' Notes

**History and Physical:**54-year-old male with plaque psoriasis for 20 years, managed by a dermatologist with methotrexate once weekly. Stopped methotrexate 6 weeks ago after being laid off and losing his drug coverage. Since then drinks “6 to 8 beers a day.” Two weeks ago a walk-in clinic prescribed a short course of oral prednisone for a flare; he finished it 5 days ago. Father has psoriasis. Weight 92 kg.

Phase 1 — Emergency Department, 2140.

➤ Complete the following sentences by choosing from the lists of options.

The client’s hypotension and tachycardia are most likely caused by

Select...▼ as evidenced by

Select...▼ . His

temperature of 35.6 °C is most likely due to

Select...▼ .

The bilateral ankle edema is most likely related to

Select...▼ . The flare was most likely triggered by

Select...▼ .

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Question 2

COMPLETE

Case Study Screen 2 of 6

The nurse is caring for a 54-year-old male client with plaque psoriasis progressing to generalized exfoliative dermatitis (erythroderma).

History and Physical

Nurses' Notes

**History and Physical:**54-year-old male with plaque psoriasis for 20 years, managed by a dermatologist with methotrexate once weekly. Stopped methotrexate 6 weeks ago after being laid off and losing his drug coverage. Since then drinks “6 to 8 beers a day.” Two weeks ago a walk-in clinic prescribed a short course of oral prednisone for a flare; he finished it 5 days ago. Father has psoriasis. Weight 92 kg.

Phase 1 — Emergency Department, 2140.

➤ Complete the following sentences by choosing from the lists of options.

The client’s hypotension and tachycardia are most likely caused by

fluid loss through the widely inflamed, shedding skin

 as evidenced by 

the orthostatic blood pressure drop, dry oral mucosa, and low, concentrated urine output

 . His

temperature of 35.6 °C is most likely due to

heat loss through widespread, dilated skin blood vessels

 . The bilateral ankle edema is most likely related to 

a low serum albumin from protein loss through the skin

 . The flare was most

likely triggered by

stopping methotrexate, finishing a course of oral prednisone, heavy alcohol use, and stress

 .

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CORRECT

Score: 4 / 4 points

**Rationale:**

In erythroderma the skin barrier is almost completely absent, so the patient loses fluid rapidly through the skin. The orthostatic drop, dry mucosa, and low, concentrated urine output confirm volume depletion; a normal SpO2 and clear lungs argue against a cardiopulmonary cause, and there is no allergen exposure suggesting anaphylaxis. Inflamed, vasodilated skin over 90% of the body loses heat rapidly; the chapter directs frequent temperature monitoring with warming or cooling as needed. The client has not stopped drinking long enough for significant withdrawal, and alcohol withdrawal tends to raise, not lower, temperature. An albumin of 26 g/L shows protein loss through scaling and exudation; low oncotic pressure lets fluid shift into tissues even while the client is intravascularly depleted, and IV fluids have not yet been given. The chapter lists stress and excessive alcohol as psoriasis triggers and notes psoriasis is a contributing cause of erythroderma. Abruptly stopping systemic therapy and withdrawal of systemic corticosteroids are well-recognized triggers of erythrodermic flares. A streptococcal infection classically triggers guttate psoriasis; detergents trigger contact dermatitis, not this presentation.

Question 3

NOT COMPLETE

Case Study Screen 3 of 6

The nurse is caring for a 54-year-old male client with plaque psoriasis progressing to generalized exfoliative dermatitis (erythroderma).

History and Physical

Nurses' Notes

**History and Physical:**54-year-old male with plaque psoriasis for 20 years, managed by a dermatologist with methotrexate once weekly. Stopped methotrexate 6 weeks ago after being laid off and losing his drug coverage. Since then drinks “6 to 8 beers a day.” Two weeks ago a walk-in clinic prescribed a short course of oral prednisone for a flare; he finished it 5 days ago. Father has psoriasis. Weight 92 kg.

Phase 1 — Emergency Department, 2140.

➤ Complete the following sentences by choosing from the lists of options.

The priority nursing hypothesis at this time is

Select...

▼

 because

Select...

▼

. Once this is addressed, the nurse's next priority is 

Select...

▼

 .

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Question 3

COMPLETE

Case Study Screen 3 of 6

The nurse is caring for a 54-year-old male client with plaque psoriasis progressing to generalized exfoliative dermatitis (erythroderma).

History and Physical

Nurses' Notes

**History and Physical:**54-year-old male with plaque psoriasis for 20 years, managed by a dermatologist with methotrexate once weekly. Stopped methotrexate 6 weeks ago after being laid off and losing his drug coverage. Since then drinks “6 to 8 beers a day.” Two weeks ago a walk-in clinic prescribed a short course of oral prednisone for a flare; he finished it 5 days ago. Father has psoriasis. Weight 92 kg.

Phase 1 — Emergency Department, 2140.

➤ Complete the following sentences by choosing from the lists of options.

The priority nursing hypothesis at this time is

risk for hemodynamic instability related to fluid and electrolyte loss

▼

because

the skin barrier is almost completely absent, allowing ongoing loss of fluid, protein, electrolytes, and heat

. Once this is addressed, the nurse's next priority is 

impaired skin integrity

▼

.

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CORRECT

Score: 3 / 3 points

**Rationale:**

For ordinary psoriasis, impaired skin integrity is the priority. Once the disease becomes erythroderma, the priority shifts to hemodynamic instability, because the skin barrier is almost completely absent -- this is the key clinical-judgment shift in this case. That widespread loss of the skin barrier is the mechanism that makes erythroderma life-threatening; the other options are true facts but do not explain the immediate danger. Impaired skin integrity is the next hypothesis in erythroderma. Body image, knowledge, and coping are real needs for this client (he is tearful and isolating) and are addressed once he is physiologically stable, in keeping with the priority of physiological needs.

Question 4

NOT COMPLETE

Case Study Screen 4 of 6

The nurse is caring for a 54-year-old male client with plaque psoriasis progressing to generalized exfoliative dermatitis (erythroderma).

History and Physical

Nurses' Notes

**History and Physical:**54-year-old male with plaque psoriasis for 20 years, managed by a dermatologist with methotrexate once weekly. Stopped methotrexate 6 weeks ago after being laid off and losing his drug coverage. Since then drinks “6 to 8 beers a day.” Two weeks ago a walk-in clinic prescribed a short course of oral prednisone for a flare; he finished it 5 days ago. Father has psoriasis. Weight 92 kg.

Phase 1 — Emergency Department, 2140.

- Which interventions should the nurse include in the plan of care for the first 24 hours? **Select all that apply.**
- ☐ 1. Administer IV isotonic fluids as prescribed
- ☐ 2. Replace potassium and magnesium as prescribed, with cardiac monitoring
- ☐ 3. Keep the room warm, apply warm blankets, and monitor temperature frequently
- ☐ 4. Maintain strict intake and output and obtain a daily weight
- ☐ 5. Provide lukewarm oatmeal soaks followed by a bland, oil-based emollient
- ☐ 6. Administer the prescribed oral antihistamine, preferably at bedtime
- ☐ 7. Monitor for alcohol withdrawal using a validated scale (e.g., CIWA-Ar)
- ☐ 8. Restrict oral fluids to 1 L/day to reduce the ankle edema
- ☐ 9. Apply coal tar ointment to all reddened skin
- ☐ 10. Arrange an ultraviolet phototherapy session for tomorrow
- ☐ 11. Use hot water and a washcloth to scrub off loose scale

Submit

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Question 4

COMPLETE

Case Study Screen 4 of 6

The nurse is caring for a 54-year-old male client with plaque psoriasis progressing to generalized exfoliative dermatitis (erythroderma).

History and Physical

Nurses' Notes

**History and Physical:**54-year-old male with plaque psoriasis for 20 years, managed by a dermatologist with methotrexate once weekly. Stopped methotrexate 6 weeks ago after being laid off and losing his drug coverage. Since then drinks “6 to 8 beers a day.” Two weeks ago a walk-in clinic prescribed a short course of oral prednisone for a flare; he finished it 5 days ago. Father has psoriasis. Weight 92 kg.

Phase 1 — Emergency Department, 2140.

- Which interventions should the nurse include in the plan of care for the first 24 hours? **Select all that apply.**
- ☒ 1. Administer IV isotonic fluids as prescribed
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- ☒ 3. Keep the room warm, apply warm blankets, and monitor temperature frequently
- ☒ 4. Maintain strict intake and output and obtain a daily weight
- ☒ 5. Provide lukewarm oatmeal soaks followed by a bland, oil-based emollient
- ☒ 6. Administer the prescribed oral antihistamine, preferably at bedtime
- ☒ 7. Monitor for alcohol withdrawal using a validated scale (e.g., CIWA-Ar)
- ☐ 8. Restrict oral fluids to 1 L/day to reduce the ankle edema
- ☐ 9. Apply coal tar ointment to all reddened skin
- ☐ 10. Arrange an ultraviolet phototherapy session for tomorrow
- ☐ 11. Use hot water and a washcloth to scrub off loose scale

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CORRECT

Score: 7 / 7 points

**Rationale:**

Adequate hydration through IV fluid replacement maintains hemodynamic stability. Potassium, magnesium, and calcium are replaced when indicated; low potassium and magnesium raise dysrhythmia risk, so cardiac monitoring is prudent. Temperature monitoring with warming measures as needed addresses impaired thermoregulation. Strict intake and output and daily weight track the response to fluid replacement and ongoing losses. Oatmeal baths are recommended for erythroderma, and oil-based emollients are recommended for psoriasis; lukewarm water avoids further vasodilation and heat loss. Oral antihistamines (for example, diphenhydramine) are listed for itching in erythroderma; bedtime dosing uses the sedating effect, with supervision for getting up because of orthostatic hypotension. He reports 6 to 8 drinks a day; withdrawal can begin within 6 to 24 hours of the last drink and worsens hemodynamic instability. The edema comes from low albumin while the client is intravascularly depleted; restricting fluids would worsen hypotension and kidney injury. Coal tar is a treatment for mild, localized psoriasis; on inflamed, broken skin over 90% of the body it is irritating and can worsen erythroderma. Ultraviolet phototherapy is used for more

Question 5

NOT COMPLETE

Case Study Screen 5 of 6

The nurse is caring for a 54-year-old male client with plaque psoriasis progressing to generalized exfoliative dermatitis (erythroderma).

History and Physical

Nurses' Notes

Nurses' Notes Day 3

Screening Results

**History and Physical:**54-year-old male with plaque psoriasis for 20 years, managed by a dermatologist with methotrexate once weekly. Stopped methotrexate 6 weeks ago after being laid off and losing his drug coverage. Since then drinks “6 to 8 beers a day.” Two weeks ago a walk-in clinic prescribed a short course of oral prednisone for a flare; he finished it 5 days ago. Father has psoriasis. Weight 92 kg.

Phase 2 — Medical Unit, Hospital Day 3.

- For each nursing action, click to specify whether it is **indicated** or **contraindicated** for the client on hospital day 3.

| Finding / Action                                                                                                       | Indicated             | Contraindicated       |
|------------------------------------------------------------------------------------------------------------------------|-----------------------|-----------------------|
| Confirm that the TB and hepatitis screening results are documented before the first infliximab infusion                | <input type="radio"/> | <input type="radio"/> |
| Monitor vital signs and observe for an infusion reaction (fever, chills, dyspnea, hives) during and after the infusion | <input type="radio"/> | <input type="radio"/> |
| Teach the client to report fever, cough, night sweats, or signs of skin infection promptly                             | <input type="radio"/> | <input type="radio"/> |
| Advise the client to get any overdue live vaccines (e.g., MMR) now that infusions are starting                         | <input type="radio"/> | <input type="radio"/> |
| Explain that the infusions can be stopped once his skin clears                                                         | <input type="radio"/> | <input type="radio"/> |
| Refer the client to a social worker for drug-coverage options and to a psoriasis support group or counselling          | <input type="radio"/> | <input type="radio"/> |
| Teach stress-reduction strategies and support his plan to reduce alcohol intake                                        | <input type="radio"/> | <input type="radio"/> |
| Encourage long, hot showers to soften and remove scale                                                                 | <input type="radio"/> | <input type="radio"/> |

Note: Each row must have only 1 response option selected.

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Question 5

COMPLETE

Case Study Screen 5 of 6

The nurse is caring for a 54-year-old male client with plaque psoriasis progressing to generalized exfoliative dermatitis (erythroderma).

History and Physical

Nurses' Notes

Nurses' Notes Day 3

Screening Results

**History and Physical:**54-year-old male with plaque psoriasis for 20 years, managed by a dermatologist with methotrexate once weekly. Stopped methotrexate 6 weeks ago after being laid off and losing his drug coverage. Since then drinks “6 to 8 beers a day.” Two weeks ago a walk-in clinic prescribed a short course of oral prednisone for a flare; he finished it 5 days ago. Father has psoriasis. Weight 92 kg.

Phase 2 — Medical Unit, Hospital Day 3.

- For each nursing action, click to specify whether it is **indicated** or **contraindicated** for the client on hospital day 3.

| Finding / Action                                                                                                       | Indicated                        | Contraindicated                  |
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| Monitor vital signs and observe for an infusion reaction (fever, chills, dyspnea, hives) during and after the infusion | <input checked="" type="radio"/> | <input type="radio"/>            |
| Teach the client to report fever, cough, night sweats, or signs of skin infection promptly                             | <input checked="" type="radio"/> | <input type="radio"/>            |
| Advise the client to get any overdue live vaccines (e.g., MMR) now that infusions are starting                         | <input type="radio"/>            | <input checked="" type="radio"/> |
| Explain that the infusions can be stopped once his skin clears                                                         | <input type="radio"/>            | <input checked="" type="radio"/> |
| Refer the client to a social worker for drug-coverage options and to a psoriasis support group or counselling          | <input checked="" type="radio"/> | <input type="radio"/>            |
| Teach stress-reduction strategies and support his plan to reduce alcohol intake                                        | <input checked="" type="radio"/> | <input type="radio"/>            |
| Encourage long, hot showers to soften and remove scale                                                                 | <input type="radio"/>            | <input checked="" type="radio"/> |

Note: Each row must have only 1 response option selected.

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CORRECT

Score: 8 / 8 points

**Rationale:**

For systemic and biologic therapy (infliximab is named), the nurse monitors for serious infections such as tuberculosis and hepatitis. TNF inhibitors can reactivate latent TB and hepatitis B, so screening must be confirmed first. Infliximab can cause infusion reactions. Suppressed immunity makes infection more likely and directs teaching about worsening redness, swelling, or discharge. Live vaccines are avoided during biologic immunosuppression; needed vaccines should be given before therapy starts, per prescriber. Psoriasis is chronic with remission and relapse; the nurse reinforces adherence rather than suggesting therapy can simply be stopped once the skin clears -- stopping therapy is what precipitated this admission. Addressing the root cause of non-adherence (lost drug coverage) and the psychosocial impact matters: support groups and social work are appropriate for coping and financial concerns. Education to avoid triggers, including stress and excessive alcohol, and stress-reduction techniques are directed. Heat and prolonged water exposure dry and irritate the skin and promote heat and fluid loss; lukewarm soaks followed by emollients are appropriate instead.

Question 6

NOT COMPLETE

Case Study Screen 6 of 6

The nurse is caring for a 54-year-old male client with plaque psoriasis progressing to generalized exfoliative dermatitis (erythroderma).

History and Physical

Nurses' Notes

Nurses' Notes Day 3

Screening Results

Nurses' Notes Day 7

**History and Physical:**54-year-old male with plaque psoriasis for 20 years, managed by a dermatologist with methotrexate once weekly. Stopped methotrexate 6 weeks ago after being laid off and losing his drug coverage. Since then drinks “6 to 8 beers a day.” Two weeks ago a walk-in clinic prescribed a short course of oral prednisone for a flare; he finished it 5 days ago. Father has psoriasis. Weight 92 kg.

Phase 3 — Medical Unit, Hospital Day 7.

- For each finding on hospital day 7, click to specify whether it indicates that the plan of care has been **effective** or **not effective**.

| Finding / Action                                                                                 | Effective             | Not effective         |
|--------------------------------------------------------------------------------------------------|-----------------------|-----------------------|
| BP 124/78 mm Hg and HR 82/min, with no orthostatic change                                        | <input type="radio"/> | <input type="radio"/> |
| Temperature 36.8 °C                                                                              | <input type="radio"/> | <input type="radio"/> |
| Potassium 4.1 mmol/L; magnesium 0.84 mmol/L                                                      | <input type="radio"/> | <input type="radio"/> |
| Erythema reduced to about 40% of the body; less peeling; itch rated 3/10                         | <input type="radio"/> | <input type="radio"/> |
| Excoriation on the right shin now has a honey-coloured crust with surrounding redness and warmth | <input type="radio"/> | <input type="radio"/> |
| Alcohol withdrawal scores 0–2 for the past 72 hours                                              | <input type="radio"/> | <input type="radio"/> |
| Client states: “If my skin stays clear, I'll just skip the next infusion.”                       | <input type="radio"/> | <input type="radio"/> |

Note: Each row must have only 1 response option selected.

Submit

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Question 6

COMPLETE

Case Study Screen 6 of 6

The nurse is caring for a 54-year-old male client with plaque psoriasis progressing to generalized exfoliative dermatitis (erythroderma).

History and Physical

Nurses' Notes

Nurses' Notes Day 3

Screening Results

Nurses' Notes Day 7

**History and Physical:**54-year-old male with plaque psoriasis for 20 years, managed by a dermatologist with methotrexate once weekly. Stopped methotrexate 6 weeks ago after being laid off and losing his drug coverage. Since then drinks “6 to 8 beers a day.” Two weeks ago a walk-in clinic prescribed a short course of oral prednisone for a flare; he finished it 5 days ago. Father has psoriasis. Weight 92 kg.

Phase 3 — Medical Unit, Hospital Day 7.

- For each finding on hospital day 7, click to specify whether it indicates that the plan of care has been **effective** or **not effective**.

| Finding / Action                                                                                 | Effective                        | Not effective                    |
|--------------------------------------------------------------------------------------------------|----------------------------------|----------------------------------|
| BP 124/78 mm Hg and HR 82/min, with no orthostatic change                                        | <input checked="" type="radio"/> | <input type="radio"/>            |
| Temperature 36.8 °C                                                                              | <input checked="" type="radio"/> | <input type="radio"/>            |
| Potassium 4.1 mmol/L; magnesium 0.84 mmol/L                                                      | <input checked="" type="radio"/> | <input type="radio"/>            |
| Erythema reduced to about 40% of the body; less peeling; itch rated 3/10                         | <input checked="" type="radio"/> | <input type="radio"/>            |
| Excoriation on the right shin now has a honey-coloured crust with surrounding redness and warmth | <input type="radio"/>            | <input checked="" type="radio"/> |
| Alcohol withdrawal scores 0–2 for the past 72 hours                                              | <input checked="" type="radio"/> | <input type="radio"/>            |
| Client states: “If my skin stays clear, I'll just skip the next infusion.”                       | <input type="radio"/>            | <input checked="" type="radio"/> |

Note: Each row must have only 1 response option selected.

Submit

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CORRECT

Score: 7 / 7 points

**Rationale:**

Stable blood pressure and heart rate meet the expected outcome of vital signs within normal limits without fluid imbalance. A normal temperature shows thermoregulation restored. Normal potassium and magnesium show no electrolyte disturbance. The chapter expects the skin to begin healing with decreased peeling and itching. A honey-coloured crust with surrounding redness and warmth is a classic sign of secondary bacterial infection (impetigo-like); the chapter notes yellow crusts over excoriations signal infection -- notify the provider; topical mupirocin is commonly prescribed, and infection risk is higher on a biologic. Low withdrawal scores show withdrawal was prevented or managed. Planning to skip the next infusion once the skin clears shows the adherence teaching has not been understood; the chapter directs the nurse to evaluate education and reinforce continuing treatment.